



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ثابت سيد عبدالجواد جويده
المحافظة : الجيزة
الحى / المدينة : سته اكتوبر
رقم البطاقة : 25311142101092 رقم التليفون : 01140205353
العنوان : 0
العمر (تاريخ الميلاد) : 14-11-1953 المهنة : بدون عمل
تاريخ تسجيل الإستمارة : 03-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
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التوقيع: -----
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التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: ثابت سيد عبد الجواد جويده
National ID: 25311142101092



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

ثابت
سيد عبدالجواد جويده
عماره ٩ مساكن الشباب ٦٣ م
٦ أكتوبر اول - الجيزه

١٩٥٣/١٠/٢٤ ٢٥٣ ١١ ١٤ ٢١ ٠١ ٩٢

KL6846964



Name: ثابت سيد عبد الجواد جويده			
MR. : 4655			
Detailed Medical Report			
		السيد	
تحتية طبية وبعد,,			
إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	
نشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 74-year-old individual with a diagnosis of luminal de-novo metastatic breast cancer involving the lungs, currently receiving Zoladex and Femara. The patient is a 72-year-old male with invasive ductal carcinoma, grade I, ER-positive, with a 5/8 PR and 8/8 ER status, who has a history of breast cancer that is locally advanced and has metastasized to the lungs. The patient has been on an aromatase inhibitor since October 2025. A CT scan performed on 17-01-2026 showed regression of the breast mass and regression of pulmonary nodules, with stable disease. The patient is today receiving Femara and Zoladex, with supportive therapy for three months. The patient has been on Zoladex and Femara for 3 months, with an annual assessment scheduled.			
تحريراً في ,			
التاريخ :		19-09-2026	
الوقت :		اسماء محمد مصطفى نصر	
الطبيب المعالج :			
يعتمد ,			

Patient Name: Thabet Sayed Abdel Gawwad Gowaidah	Date : 09/08/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 71 Year(s)
Medical ID:	Pat. ID: 110888

BIATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

- Breast density: Scattered areas of glandular tissue...Matching ACR b.
- Thickened right breast skin with large infiltrative mass is seen in the right central part of the breast reaching the posterior third.
- No detected suspicious mass density, architectural distortion or pathological micro-calcification in the left breast.
- Normal skin and nipple on left side.

By complementary breast ultrasound:

Right breast:

- Homogenous (predominantly fatty) echo-texture of right breast.
- Thickened right breast skin reaching maximum diameter of 6 mm with underlying mild interstitial edema and dilated sub-dermal lymphatics.
- Large retro areolar irregular shaped infiltrative mass lesion is seen infiltrating most of the right breast parenchyma reaching, reaching the pectoral muscle, it measures (6 x 5.5 cm).
- Few (at least 3 in number) suspicious looking level1, right axillary LNs the largest measures (10 x 8 mm), (7 x 3.5 cm).

Left breast:

- Homogenous (predominantly fatty) echo-texture of left breast.
- No detected suspicious solid or cystic masses in the left breast.
- Normal left breast skin and nipple. No detected significant left axillary LNs.

CONCLUSION:

- Right breast: Locally advanced highly suspicious looking breast mass.....St-
RADS 5 category.
- Left breast: Normal study.... BIRADS 1 Category

Much Obligated

DR. MONA ZAKY MD

professor-Breast Imaging-Mansoura University

Patient Name: Thabet Sayed Abdel Gawwad Gowaideh	Date: 27/08/2025
Exam: CT CAP+C (CT Chest Abd. Pelvis)	Age: 72 Year(s)
Medical ID: 1027/2025	Pat. ID: 110888

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

Findings:

I Chest:

- > Right breast solid lesion is seen measuring about 5.3 x 5.6 cm, seen invading the underlying pectoralis muscle.
- > Multiple pathologically enlarged right level I axillary lymph nodes, the largest measures about 1.9 x 1.4 cm.
- > Diffuse reticulonodular interstitial pulmonary changes are seen involving right lungs more appreciated in lower lung lobe manifested by:
 - o Diffuse ground glass haziness intermingled with areas of air-trapping.
 - o Interstitial coarsening with reticular infiltrations.
 - o Peri-bronchial cuffing with bronchial wall thickening.
 - o Stellate shaped blood vessels.
 - o Thickened inter-lobar septa / fissures with subpleural lines & septal lines.
- > Bilateral pulmonary nodules, the largest seen at right lower lobe measures 10 x mm.
- > Few enlarged mediastinal lymph nodes, the largest seen at prevascular space measures 13 x 4mm.
- > No evidence of pleural effusion, pleural thickening or masses seen.
- > Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- > Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- > The liver is of average size with homogenous CT texture. No definite focal lesions (on non contrast base) or contour irregularities.
- > No intra-hepatic or extra-hepatic biliary radicles dilatation.
- > Left kidney show crossed fused ectopia.
- > Bilateral nodular thickening of both adrenals.
- > Normal size & parenchymal texture of the spleen with no focal lesions seen.
- > Normal appearance of the pancreas, aorta, I.V.C.
- > Right large inguinal hernia containing fat.
- > Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- > Normal appearance of the other pelvic organs.
- > No abdominal or pelvic masses or collections.

***Scanned skeleton:** Mottled appearance of axial and appendicular skeleton.

Opinion:

Known case of right breast cancer shows:

- o Right solid lesion associated with multiple enlarged ipsilateral axillary lymph nodes.
- o Picture suggestive of part of lymphangitis carcinomatosa with multiple pulmonary nodules.
- o Left crossed fused ectopia.
- o Mottled appearance of examined bones likely metastatic deposits.

Dr. Reham Khedr, M.D.

Signature Reham Khedr
30/08/2025 3:09 PM

Patient Name : الأستاذ / ثابت محمد عبدالجواد جويده
Patient No. : 25311142101092
Path No. : 2588/25
Sex : Male
Age : 71 Y
Referring Doc :

Request No. : 13003520
Request Date : 20/08/2025
Verification Date : 25/08/2025
Financial Category : جراحات روسي - تنقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data
Right breast mass.

Nature of specimen
Tru cut biopsy, collected on 19/8/2025.

Gross examination

Multiple (3) grayish white tissue cores ranging in size from 0.5 cm up to 1.5 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination

- Histological type: Invasive duct carcinoma
- Grade (differentiation +nuclear pleomorphism+ mitosis): (1+2+2) Grade 1
- DCIS: Absent.
- Angioinvasion: Absent
- Perineural invasion: Absent
- IPT (Positive control showed appropriate reactivity) :
- ER: Strong nuclear reaction in 90 % tumor cells (Allred score 8/8)
- PR: Strong nuclear reaction in 10 % tumor cells (Allred score 5/8)
- HER2: Faint incomplete membrane reaction in >10 % of tumor cells (score 1+)
- Ki 67: 15 % nuclear reaction of tumor cells

Diagnosis

Verified by :
Dr.Heba Hasan

Printing Date : 27/08/2025 10:42:45

Page 1 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنوش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadima - Cairo, Egypt

Patient Name : ١٤٣٥ / ٢٤٣٥ / ٢٤٣٥
Patient No. : 25311142101092
Path No. : 2588/25
Sex : Male
Age : 71 Y
Referring Doc :

Request No. : 13003520
Request Date : 20/08/2025
Verification Date : 25/08/2025
Financial Category : نفقة الدولة - جوماتل روسي
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Right breast mass, Tru cut biopsy:
Invasive duct carcinoma, Grade 1
IPT:

- * ER: Positive (score 8/8).
- * PR: Positive (score 5/8).
- * HER2: (score 1+).
- * Ki67: Positive in 15% of tumor cells.

Comment: Although Her2 is Negative, Such cases (Score 1+) are predictive of response to Her2-Ab drug conjugates (ADCS) according to latest 2023 ASCO-CAP guidelines.

Signature

Dr. Walaa Ibrahim Ghoneim
Dr. Heba El-Shafeey .

Verified by :
Dr. Heba Hasan

Printing Date : 27/08/2025 10:42:45

Page 2 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadema - Cairo, Egypt

Name : Thabet Said Abdeljawad Jouida
Branch : Dokki

Date : 05-09-2025 12:04
Medical No. : 195051

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 72-year-old male, with right breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen all over the surveyed skeleton.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

UNILE

ECHO REPORT

NAME:

ثابت سعيد عبد الجواد

ID NUMBER:

١٠٧/٢٠

DATE: 28 / 8 / 2026

SEX:

AGE:

EF	34-40%	55-75%	PWT	1.2	0.6-1.1cm
VEDD	6.5	3.5-5.8cm	AO.R		3-3.7cm
VESD	5.4	2-4cm	LA	40	2-4cm
ES	16	25-45%	RV		0.7-2.3cm(supine)
SWT	1.1	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function

SWMA

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

Distorted LV Dimensions

Single E wave for CCS
Global Hypokinesis

Dilated LA

Mild MR Moderate MV
Mild PR Eccentric DV

Mild TR RSP-28

17L

DR: 

Abou Cbe

Patient's Name: Thabet Said Abdeljawad Jouida
Date: 2026-08-18
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- o Bilateral pulmonary emphysematous changes .
- o No consolidation or collapse.
- o No hilar or mediastinal masses or lymph node enlargement.
- o No pleural or pericardial collection.
- o Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- o Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- o Hypoplastic left kidney
- o Posed right kidney showed average size and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- o Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o No abdominal or pelvic lymphadenopathy.
- o No ascites.
- o Large right inguinal hernia

Opinion:

- ❖ No hepatic or pulmonary metastatic lesions.
- ❖ Hypoplastic left kidney.
- ❖ Large right inguinal hernia

Dr.Asser Ahmed

Name : Thabet Said Abdeljawad Jouida

Branch : Dokki

Ref.Physician Prof : Goustav Roussey Egypt

Date : 31-08-2026 11:44

Medical No. : 451155

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 72-year-old male with primary right breast carcinoma on hormonal therapy. No bone pain. Referred for follow-up skeletal evaluation compared to prior study dated May 9, 2025.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Mild thoracolumbar scoliosis is noted with focal uptake at the right side of the T12 vertebral body/facet complex, consistent with active degenerative spondylosis. The remainder of the axial and appendicular skeleton shows symmetric physiological tracer distribution without suspicious focal lesions compared to the prior study.
- Mild symmetric degenerative uptake is seen in peripheral joints.
- Bilateral native renal beds are empty. A single functioning renal structure is visualized in the right iliac fossa/pelvis, consistent with a renal transplant.

Impression: Stable compared to prior study (May 9, 2025).

- No evidence of skeletal metastases on the current study.
- Follow up as clinically indicated would be recommended.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

جمهورية مصر العربية
بطاقة تحقق الشخصية

ثابت
سيد عبدالجواد جويده
عشارو ٩ مساكن الشباب ٣ تم
٦ أكتوبر الأول - الجيزة

١٩٥٣/٨

٢٥٣ ١١ ١٤ ٢١ ٢١.٠٩٢

KL6846964



٢٠٢٤/٣ ٢٥٢١١٤٢١٠١٠٩٢
بيون عمل
مترج مسلم
نفر
البطاقة سارية حتى ٢٠٣١/٣/٠٨



